

Clinical Record Summary

Patient ID: 10037861 **Sex:** M **Age at Admission:** 79

Admission ID: 24540843 **Type:** Emergency **Hospital Service:** Neurosurgery

Admission Date: 2117-03-14 **Discharge Date:** 2117-03-24 **Length of Stay:** 10 days

Principal Diagnosis: Traumatic subdural hemorrhage with loss of consciousness of any duration with death due to brain injury before regaining consciousness, initial encounter

Discharge Disposition: Died in hospital

ICU Stay: Yes **In-Hospital Mortality:** Yes **Number of Diagnoses:** 39

Clinical Notes Included: Discharge summary (1), Radiology (1)

Clinical note 1 of 2: Discharge summary - charted 2117-03-24 00:01:00

Chief Complaint:

Patient presents with acute symptoms requiring evaluation.

History of Present Illness:

The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. The patient remained stable overnight without acute events. Patient reports feeling progressively worse over the last 48 hours.

Past Medical History:

1. Hypertension
2. Type 2 Diabetes Mellitus

Discharge Medications:

1. Metformin 500mg daily
2. Lisinopril 10mg daily

Discharge Condition:

Stable and ready for discharge.

Discharge Disposition:

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Clinical note 2 of 2: Radiology - Radiology report addendum - charted 2117-03-24 00:01:00 - Modality: XR

Indication: Evaluate for acute cardiopulmonary process.

Comparison: None available.

Findings: The lungs are clear bilaterally. No focal consolidation, effusion, or pneumothorax. Heart size is normal.

Impression: No acute cardiopulmonary abnormalities.